REQUEST FOR PROPOSAL (RFP)

Synthetic Test Document • Complex Multi-Module Healthcare Platform • Version 2.3

IMPORTANT: This is a fictional document created solely for software testing. All organizations, names, dates, volumes, and requirements are invented. It is not a real client RFP.

Document Control

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1. Background and Business Objectives

Northstar Regional Care Network (NRCN) is a fictional healthcare group operating 14 outpatient clinics and 3 small hospitals across three Indian states. The organization wants to consolidate fragmented appointment, patient-record, billing, and clinician-workflow systems into a single digital platform. The proposed platform must support web access for staff and patients, with mobile experiences for patients and clinicians.

The primary goals are to reduce appointment friction, improve care-team visibility, modernize patient communications, and provide management with consistent operational reporting. The RFP does not authorize the vendor to make clinical decisions or recommend diagnoses.

2. Scope of Work

The selected vendor is expected to design, build, integrate, test, deploy, and provide post-launch support for the platform. The scope includes the following workstreams:

WS-01 — Patient access: registration, identity verification, appointment search, booking, rescheduling, cancellation, reminders, wait-list management, and accessibility support.

WS-02 — Clinician workspace: patient timeline, encounter notes, medication list display, document attachments, referral tracking, and role-based access.

WS-03 — Billing and payments: invoices, refunds, payment status, insurer claim references, and reconciliation exports. The platform must not store raw card numbers or CVV values.

WS-04 — Integrations: existing hospital information system (HIS), laboratory information system (LIS), SMS provider, email provider, payment gateway, and a national health identifier service where legally and technically applicable.

WS-05 — Operations and analytics: clinic-level dashboards, appointment no-show reporting, audit trails, configurable service metrics, and CSV exports.

WS-06 — Migration: migration of selected patient demographics and appointment history from two legacy databases. The source schemas and data quality profile will be provided after vendor selection.

Out of scope: autonomous diagnosis, treatment recommendations, clinical triage decisions, and training a medical foundation model. Any AI-assisted administrative features require explicit approval and human oversight.

3. Functional Requirements

The following requirements were collected from workshops, email threads, and an older product brief. Some statements may be incomplete or inconsistent; bidders should identify questions rather than silently resolve them.

FR-001 [Must] — Patients must search for clinicians by specialty, clinic, language, and available appointment slot.

FR-002 [Must] — Patients may book, reschedule, and cancel appointments. Cancellation cut-off rules must be configurable by clinic.

FR-003 [Should] — The system should send appointment reminders by SMS and email. A product note says WhatsApp reminders are also desired, subject to consent and provider approval.

FR-004 [Must] — Clinicians can view the patient's longitudinal timeline, including prior encounters and attached documents, subject to authorization.

FR-005 [Must] — A clinician may save encounter notes as drafts and finalize them after review. Drafts must not appear as finalized clinical records.

FR-006 [Must] — Reception staff can check patients in and mark appointments as arrived, late, or did-not-attend.

FR-007 [Must] — Billing staff can issue invoices, record offline payments, initiate refunds, and export reconciliation data.

FR-008 [Must] — The platform shall integrate with the existing HIS. The current HIS vendor has not been selected for disclosure at this stage.

FR-009 [Must] — The platform should exchange laboratory orders and results with the LIS using the interface supported by the incumbent vendor.

FR-010 [Must] — Administrators can create roles and assign permissions by clinic, department, and data category.

FR-011 [Should] — Users must be able to export permitted operational reports in CSV format.

FR-012 [Must] — Patients can download a copy of their own available records after identity verification.

FR-013 [Should] — The system should support English and Hindi at launch; regional-language expansion may be considered later.

FR-014 [Must] — A manager dashboard shall display appointment volume, cancellation rate, no-show rate, and average waiting time.

FR-015 [Must] — The vendor should propose a method to migrate patient demographics and appointment history with reconciliation and rollback.

FR-016 [Must] — Support staff can merge duplicate patient profiles only after a documented verification workflow and audit event.

FR-017 [Must] — The platform should support online payment and payment status callbacks from the selected payment provider.

FR-018 [Must] — A patient can request correction of inaccurate demographic details. Changes to finalized clinical notes require a separate amendment workflow.

FR-019 [Should] — The solution should provide configurable notifications for appointment changes and service disruptions.

FR-020 [Must] — The product brief states that clinicians need 'instant access' to patient history, but no measurable latency target is supplied.

FR-021 [Must] — The platform shall support 10,000 concurrent users during seasonal campaign periods.

FR-022 [Must] — A meeting note says the first release is needed in 16 weeks. The delivery section later states that production launch must occur within 12 weeks of contract signature.

FR-023 [Must] — The mobile application must be available on Android and iOS. Another section says a responsive web experience may be accepted instead of native mobile apps if the total cost is lower.

FR-024 [Must] — All clinics should use a shared patient identifier. The migration note says legacy IDs must remain visible and searchable.

FR-025 [Must] — The platform shall support integration with the national health identifier service, where applicable and approved by the organization.

FR-026 [Should] — Users should be able to upload PDF, JPG, and PNG documents. Maximum file size and malware-scanning requirements are not defined.

FR-027 [Should] — The system should support appointment wait lists and automatically offer a freed slot to eligible patients.

FR-028 [Must] — The RFP asks for a 'zero downtime migration' while also allowing a planned maintenance window during cutover.

4. Non-Functional and Operational Requirements

NFR-001 — Availability target: 99.9% monthly, excluding approved maintenance. The contract appendix instead mentions 99.5%.

NFR-002 — Patient-facing pages should load in under 2 seconds under normal conditions. No percentile, geography, or device profile is stated.

NFR-003 — The system must encrypt data in transit and at rest. Specific algorithms, key ownership, and rotation intervals are not specified.

NFR-004 — Administrative and clinical access must be auditable. Audit retention duration and tamper-evidence requirements are unspecified.

NFR-005 — The platform should comply with applicable Indian privacy and healthcare requirements. The buyer expects the vendor to identify relevant obligations and confirm applicability with counsel.

NFR-006 — Recovery Point Objective (RPO) and Recovery Time Objective (RTO) are not stated.

NFR-007 — Production data must not be used to train public models or shared with unauthorized third parties.

NFR-008 — The solution should be accessible to users with disabilities. No formal conformance target or test method is specified.

NFR-009 — Logs must not contain full payment credentials. Treatment of patient identifiers in application logs needs a documented policy.

NFR-010 — The platform must support three regions. It is unclear whether this means cloud availability zones, disaster-recovery locations, or application deployment regions.

NFR-011 — The platform shall support 99.99% availability during clinic operating hours, according to a workshop note.

NFR-012 — Performance tests should cover 10,000 concurrent users, but the expected request mix, ramp-up period, and test duration are not defined.

5. Integration and Data Context

The buyer currently operates two legacy patient databases, an HIS, an LIS, a third-party payment gateway, an SMS provider, and an email service. Vendor names, API documentation, message formats, rate limits, authentication mechanisms, and sandbox access will be shared only after a mutual confidentiality agreement and shortlist approval.

An internal architecture sketch references HL7 v2 for some laboratory messages. A separate email says the LIS vendor may support FHIR APIs for selected workflows. The buyer has not confirmed which interfaces are available in production or whether an integration engine already exists.

The migration scope includes approximately 1.8 million patient records and 6.5 million historical appointment rows, but these figures are preliminary estimates from an operations spreadsheet. The number of duplicate records, missing values, invalid identifiers, and inconsistent date formats has not been profiled.

The RFP mentions a preferred cloud provider but does not identify it. A procurement note says the buyer may consider a different provider if data residency, security, support, and total cost requirements are met.

6. Security, Privacy, and Governance

The vendor must propose an access-control model, audit strategy, secrets management, secure SDLC, vulnerability management, backup approach, incident response process, and security testing plan. The buyer expects least-privilege access and separation between production and non-production environments.

Production patient data must not be placed in development environments unless explicitly authorized and appropriately protected. Test data should be synthetic or de-identified. The vendor must explain how backups, support access, observability tools, and third-party integrations will avoid unnecessary exposure of sensitive data.

The RFP asks bidders to describe how privacy obligations will be mapped to applicable Indian law and contractual obligations. This document does not establish a definitive legal interpretation; bidders should identify legal and compliance questions for the buyer and its counsel.

7. Delivery and Acceptance

The vendor must provide discovery, solution design, implementation, integration testing, user acceptance testing, migration rehearsal, production cutover, training, and post-launch support. Acceptance criteria should include functional coverage, security findings, performance testing, migration reconciliation, audit verification, and documented rollback readiness.

The project plan must identify buyer dependencies, including timely access to SMEs, test environments, API documentation, data samples, and sign-offs. The RFP does not specify the buyer's available staffing capacity or expected response time for decisions.

The 12-week production-launch requirement and the 16-week first-release note conflict. Bidders must explicitly identify the discrepancy and state what assumptions they use in any proposed plan.

8. Commercial and Procurement Notes

The buyer requests a fixed-price proposal with milestone-based payments, but has not provided a budget range. Another procurement email asks bidders to separate implementation fees, recurring cloud/software costs, support fees, and third-party licensing. The contract duration and post-launch service-level agreement remain undecided.

Bidders must describe assumptions, exclusions, change-control process, warranty, support hours, ownership of custom code, use of open-source components, and any recurring fees. No vendor may assume that all third-party services are already licensed.

9. Contradictory or Unverified Notes

The following statements were copied from working notes and should not be treated as resolved decisions:

• “We must launch in 12 weeks.” — procurement summary.

• “The first release can be delivered in 16 weeks.” — discovery workshop notes.

• “99.9% monthly availability.” — main requirements section.

• “99.5% availability is sufficient.” — draft contract appendix.

• “Native Android and iOS applications are mandatory.” — mobile section.

• “Responsive web may be accepted if cheaper.” — finance email.

• “Zero downtime migration.” — migration objective.

• “A planned maintenance window is acceptable during cutover.” — operations note.

• “Three regions required.” — infrastructure note; meaning is undefined.

• “FHIR preferred.” — integration discussion.

• “HL7 v2 currently used for laboratory messages.” — architecture sketch.

10. Bidder Response Instructions

Please submit a response containing: (1) understanding of business goals; (2) requirement traceability; (3) proposed architecture and integration strategy; (4) migration and data-quality approach; (5) security, privacy, and governance plan; (6) testing and acceptance plan; (7) phased delivery schedule with assumptions; (8) team roles and dependencies; (9) itemized commercial assumptions; and (10) a list of questions requiring buyer clarification.

Bidders should not treat missing details as permission to invent assumptions without disclosure. State which elements are confirmed, which are inferred, and which require buyer decision. Do not propose automated clinical diagnosis or treatment recommendations; these are explicitly outside scope.

END OF SYNTHETIC RFP — FOR MVP TESTING ONLY

Suggested test targets: requirement extraction, contradictory timelines and SLAs, ambiguous mobile scope, migration risks, unspecified integrations, evidence/page traceability, privacy/security requirements, and avoidance of unsupported cost estimates.